

[PROVIDER-NAME]
[PROVIDER-NAME]
[PROVIDER-NAME] PhD MRCP
Our Ref: (T) DRF/JN/ 0518393

[DATE]
Clinic Letter : [DATE]

[PROVIDER-NAME]
[ADDRESS]
[ADDRESS]
[ADDRESS]
[POSTCODE]

Division of Unscheduled Care
Care Group - Medicine & Elderly Care Gastroenterology

[ADDRESS]
[ADDRESS]
[POSTCODE]

Telephone: [PHONE]

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Dear [PROVIDER-NAME] [NAME] , [ADDRESS]. [POSTCODE]

[ADDRESS]

I reviewed this lady today. Her colonoscopy showed a total colitis and the biopsies show mild chronic inflammatory bowel disease without specific features. The appearances at endoscopy were those of a Crohn's colitis where as on histology they would be consistent with ulcerative colitis. I think the best categorisation for the time being would be indeterminate colitis. She complains of abdominal pain. It is rare for this to be a feature of inflammatory colitis without small bowel involvement. I suspect that her pain relates as much to her multiple sclerosis as to her colitis. She should be treated with a pain-modifying agent such as amitriptyline.

With regard to the Crohn's disease, unfortunately the prescription she was given was incorrect. It should have been reducing her prednisolone by 5 milligrams weekly not daily. I have therefore advised her to increase to 35 milligrams daily for a week and then reduce by 5 milligrams per week thereafter. I have given her my standard counselling for azathioprine and an azathioprine information leaflet and have checked her TPMT level. We will review her in about three weeks' time with the TPMT with the aim of starting her on azathioprine as long-term maintenance.

With kind regards

Yours sincerely

[PROVIDER-NAME] MD FRCP
Consultant Physician/Gastroenterologist
Honorary Senior Lecturer in Medicine
Regional Specialist Adviser for Gastroenterology

cc:

[NAME]
Specialist Nurse
[ADDRESS]